PharmEasy Labs

Report for Shanawaz Pasha(67Y/M)

Tests asked Crp, Comprehensive Plus Checkup With Vitamins And Electrolytes

Test date 01 May 2025 **Report status** Complete Report

REPORT VERIFIED VERIFIED BY DOCTOR

6 STEP quality control to ensure 100% report accuracy

- Qualified and trained technicians
- Temperature-controlled containers to store samples
- Strict quality checks on samples before processing
- Regular monitoring of lab analyzers by experts
- Assured machine inspection on a daily basis
- Verified reports by qualified pathologists

25+ Years of Trust & Experience | NABL Accredited Labs | 100+ Crore Samples Processed

PharmEasy Labs

NAME : SHANAWAZ PASHA(67Y/M)
REF. BY : SELF
TEST ASKED : CRP,COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES

HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

Report Availability Summary

Note: Please refer to the table below for status of your tests.

14 Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
C-REACTIVE PROTEIN (CRP)	Ready
COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND	Ready
FASTING BLOOD SUGAR(GLUCOSE)	Ready
CHLORIDE	Ready
25-OH VITAMIN D (TOTAL)	Ready
SODIUM	Ready
VITAMIN B-12	Ready
COMPLETE URINE ANALYSIS	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
LIVER FUNCTION TESTS	Ready
IRON DEFICIENCY PROFILE	Ready
KIDPRO	Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready

PharmEasy Labs

NAME	:	SHANAWAZ PASHA(67Y/M)
REF. BY	:	SELF
TEST ASKED	:	COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP

HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)			
	H.P.L.C	**12**	**%**

Bio. Ref. Interval. :

Bio. Ref. Interval.: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	**CALCULATED**	**298**	**mg/dL**

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT)	:	01 May 2025 10:25
Sample Received on (SRT)	:	01 May 2025 12:21
Report Released on (RRT)	:	01 May 2025 13:39
Sample Type	:	EDTA Whole Blood
Labcode	:	0105071642/PE004
Barcode	:	DP697024

Dr Syeda Sumaiya MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs

NAME : SHANAWAZ PASHA(67Y/M)
REF. BY : SELF
TEST ASKED : COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP

HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	**SLS-Hemoglobin Method**	**17.8**	**g/dL**	**13.0-17.0**
Hematocrit (PCV)	**CPH Detection**	**56.3**	**%**	**40.0-50.0**
Total RBC	**HF & EI**	**6.29**	**X 10^6/µL**	**4.5-5.5**
Mean Corpuscular Volume (MCV)	Calculated	89.5	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	28.3	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	31.6	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	**Calculated**	**48**	**fL**	**39-46**
Red Cell Distribution Width (RDW - CV)	**Calculated**	**14.7**	**%**	**11.6-14**
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	209.2	-	*Refer Note below
MENTZER INDEX	Calculated	14.2	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	8.24	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	64.2	%	40-80
Lymphocytes Percentage	Flow Cytometry	23.8	%	20-40
Monocytes Percentage	Flow Cytometry	5.6	%	2-10
Eosinophils Percentage	Flow Cytometry	5.7	%	1-6
Basophils Percentage	Flow Cytometry	0.4	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	5.29	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	1.96	X 10^3 / µL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.46	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.03	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.47	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	198	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	10.8	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	14.4	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	31.9	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.21	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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Report Released on (RRT) : 01 May 2025 13:39
Sample Type : EDTA Whole Blood
Labcode : 0105071642/PE004
Barcode : DP697024

Dr Syeda Sumaiya MD(Path)

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Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs

NAME	: SHANAWAZ PASHA(67Y/M)	**HOME COLLECTION :**	
REF. BY	: SELF	NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU	
TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP		

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	71.6	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-PAP METHOD

Sample Collected on (SCT)	: 01 May 2025 10:25
Sample Received on (SRT)	: 01 May 2025 12:22
Report Released on (RRT)	: 01 May 2025 14:17
Sample Type	: FLUORIDE PLASMA
Labcode	: 0105071691/PE004
Barcode	: DR072943

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REF. BY : SELF
TEST ASKED : COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP

HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
Physical Examination				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.01	-	1.003-1.030
PH	pH indicator	5	-	5-8
Chemical Examination				
URINARY PROTEIN	**PEI**	**Present 1+(30-100 mg/dl)**	**mg/dL**	**Absent**
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
Microscopic Examination				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-5
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	1	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

Sample Collected on (SCT) : 01 May 2025 10:25
Sample Received on (SRT) : 01 May 2025 17:03
Report Released on (RRT) : 01 May 2025 19:29
Sample Type : URINE
Labcode : 0105090503/PE004
Barcode : DK601694

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REF. BY	: SELF	NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU	
TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP		

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	E.C.L.I.A	79.5	ng/mL

Bio. Ref. Interval. :-

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Sample Collected on (SCT)	: 01 May 2025 10:25
Sample Received on (SRT)	: 01 May 2025 12:22
Report Released on (RRT)	: 01 May 2025 15:48
Sample Type	: SERUM
Labcode	: 0105071692/PE004
Barcode	: DT342549

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TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP		

TEST NAME	TECHNOLOGY	VALUE	UNITS
C-REACTIVE PROTEIN (CRP)	**IMMUNOTURBIDIMETRY**	**19.2**	**mg/L**

Bio. Ref. Interval. :-

Acute phase determination : < 5 mg/L

Clinical Significance:
It's a protein present in the sera of acutely ill patients that bound cell wall C-polysaccharide of streptococcus pneumoniae and agglutinates the organisms. CRP is one of the strongest acute -phase reactants, with plasma concentrations rising up after myocardial infarction, stress, trauma, infection, inflammation, surgery, or neoplastic proliferation.

Concentrations >5 mg/L suggest the presence of an infection or inflammatory process. Concentrations are generally higher in bacterial than viral infection. The increase in peak is proportional to tissue damage. Determination of CRP is clinically useful to screen activity of inflammatory diseases such as rheumatoid arthritis; SLE;Leukemia;after surgery;to detect rejection in renal allograft recipients;to detect neonatal septicemia and meningitis. However, it is a nonspecific marker and cannot be interpreted without other clinical information.

Reference:
Tietz Textbook of clinical chemistry and molecular diagnosis fifth edition chapter 21 P538-539

Please correlate with clinical conditions.

Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

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TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP		

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	E.C.L.I.A	345	pg/mL

Bio. Ref. Interval. :-

Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

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HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	74	µg/dL
Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 **Method :** Ferrozine method without deproteinization			
TOTAL IRON BINDING CAPACITY (TIBC)	PHOTOMETRY	348	µg/dL
Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl **Method :** Spectrophotometric Assay			
% TRANSFERRIN SATURATION	CALCULATED	21	%
Bio. Ref. Interval. : 13 - 45 **Method :** Derived from IRON and TIBC values			
UNSAT.IRON-BINDING CAPACITY(UIBC)	PHOTOMETRY	274.5	µg/dL
Bio. Ref. Interval. : 162 - 368 **Method :** SPECTROPHOTOMETRIC ASSAY			

Please correlate with clinical conditions.

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HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	154	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**30**	**mg/dL**	**40-60**
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**102.61**	**mg/dL**	**< 100**
TRIGLYCERIDES	PHOTOMETRY	134	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	**CALCULATED**	**5.1**	**Ratio**	**3 - 5**
TRIG / HDL RATIO	**CALCULATED**	**4.45**	**Ratio**	**< 3.12**
LDL / HDL RATIO	CALCULATED	3.4	Ratio	1.5-3.5
HDL / LDL RATIO	**CALCULATED**	**0.29**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	CALCULATED	123.5	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	26.76	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	118.69	U/L	45-129
BILIRUBIN - TOTAL	**PHOTOMETRY**	**2.43**	**mg/dL**	**0.3-1.2**
BILIRUBIN -DIRECT	**PHOTOMETRY**	**0.45**	**mg/dL**	**< 0.3**
BILIRUBIN (INDIRECT)	**CALCULATED**	**1.98**	**mg/dL**	**0-0.9**
GAMMA GLUTAMYL TRANSFERASE (GGT)	**PHOTOMETRY**	**164.5**	**U/L**	**< 55**
ASPARTATE AMINOTRANSFERASE (SGOT)	**PHOTOMETRY**	**50.8**	**U/L**	**< 35**
ALANINE TRANSAMINASE (SGPT)	**PHOTOMETRY**	**73.8**	**U/L**	**< 45**
SGOT / SGPT RATIO	CALCULATED	0.69	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	6.83	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	3.31	gm/dL	3.2-4.8
SERUM GLOBULIN	**CALCULATED**	**3.52**	**gm/dL**	**2.5-3.4**
SERUM ALB/GLOBULIN RATIO	CALCULATED	0.94	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS
SODIUM	**I.S.E - INDIRECT**	**134.32**	**mmol/L**

Bio. Ref. Interval. :

Adults: 136-145 mmol/l

Method : ION SELECTIVE ELECTRODE - INDIRECT

CHLORIDE	**I.S.E - INDIRECT**	**97.84**	**mmol/L**

Bio. Ref. Interval. :

ADULTS: 98-107 MMOL/L

Clinical Significance :
An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).

Method : ION SELECTIVE ELECTRODE - INDIRECT

Please correlate with clinical conditions.

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TEST ASKED : COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP

HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	**PHOTOMETRY**	**20.7**	**mg/dL**	**7.94 - 20.07**
CREATININE - SERUM	**PHOTOMETRY**	**1.61**	**mg/dL**	**0.72-1.18**
BUN / Sr.CREATININE RATIO	CALCULATED	12.86	Ratio	9:1-23:1
UREA (CALCULATED)	**CALCULATED**	**44.3**	**mg/dL**	**Adult : 17-43**
UREA / SR.CREATININE RATIO	CALCULATED	27.51	Ratio	< 52
CALCIUM	PHOTOMETRY	9	mg/dL	8.8-10.6
URIC ACID	**PHOTOMETRY**	**8.1**	**mg/dL**	**4.2 - 7.3**

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Sample Collected on (SCT) : 01 May 2025 10:25
Sample Received on (SRT) : 01 May 2025 12:22
Report Released on (RRT) : 01 May 2025 15:48
Sample Type : SERUM
Labcode : 0105071692/PE004
Barcode : DT342549

Dr Syeda Sumaiya MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs

NAME	:	SHANAWAZ PASHA(67Y/M)
REF. BY	:	SELF
TEST ASKED	:	COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP

HOME COLLECTION :
NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	**E.C.L.I.A**	**77**	**ng/dL**	**80-200**
TOTAL THYROXINE (T4)	E.C.L.I.A	4.82	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	1.29	µIU/mL	0.54-5.30

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Fully Automated Electrochemiluminescence Compititive Immunoassay
T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer :

Results should always be interpreted using the reference range provided by the laboratory that performed the test.
Different laboratories do tests using different technologies, methods and using different reagents which may cause difference
In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports.
To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Sample Collected on (SCT)	:	01 May 2025 10:25
Sample Received on (SRT)	:	01 May 2025 12:22
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Sample Type	:	SERUM
Labcode	:	0105071692/PE004
Barcode	:	DT342549

Dr Syeda Sumaiya MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs

NAME	: SHANAWAZ PASHA(67Y/M)	**HOME COLLECTION :**	
REF. BY	: SELF	NO-20/13 5TH CROSS CHINNAPPA GARDEN BENSON TOWN BANGALORE BENSON TOWN BENGALURU	
TEST ASKED	: COMPREHENSIVE PLUS CHECKUP WITH VITAMINS AND ELECTROLYTES,CRP		

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	**CALCULATED**	**47**	**mL/min/1.73 m2**

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

~~ End of report ~~

Sample Collected on (SCT)	: 01 May 2025 10:25
Sample Received on (SRT)	: 01 May 2025 12:22
Report Released on (RRT)	: 01 May 2025 15:48
Sample Type	: SERUM
Labcode	: 0105071692/PE004
Barcode	: DT342549

Dr Syeda Sumaiya MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

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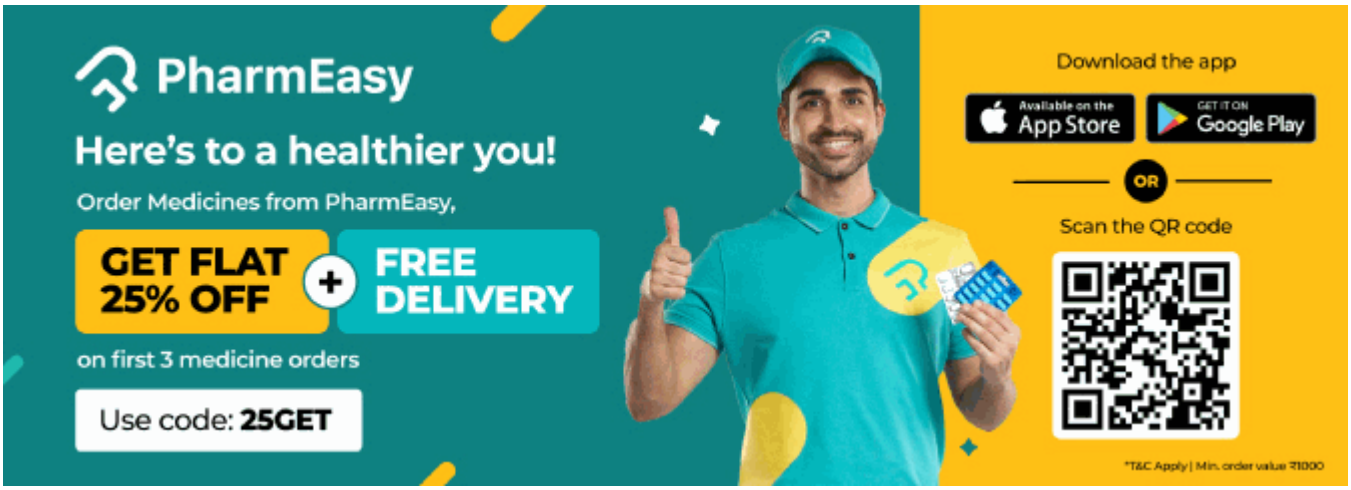
CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Docon Technologies Private Limited,Thyrocare Technologies Limited and its employees/representatives do not assume any liability,responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- **Name -** The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.By -** The name of the doctor who has recommended testing as declared by the client.
- **Labcode -** This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode -** This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCT -** Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT -** Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT -** Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range -** Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- For suggestions, complaints or feedback, write to us at grievance-office@docon.co.in or call us on 7022000900.